

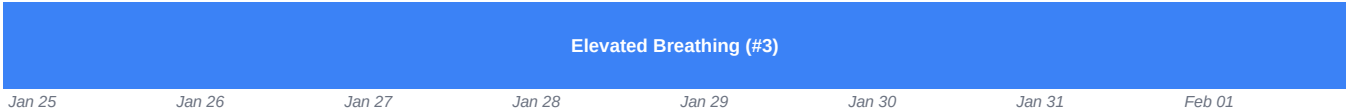
CLINICAL INTELLIGENCE TREND REPORT

**YELLOW: Closer Observation
Suggested**

Patient ID: Wimberley **Period:** January 25 to February 1, 2024 **Report Date:** February 2, 2024 **Coverage:** 117/192h (60.9%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Patient clinical status over 8 days from Jan 25 to Feb 01 ■ HR ■ Breathing

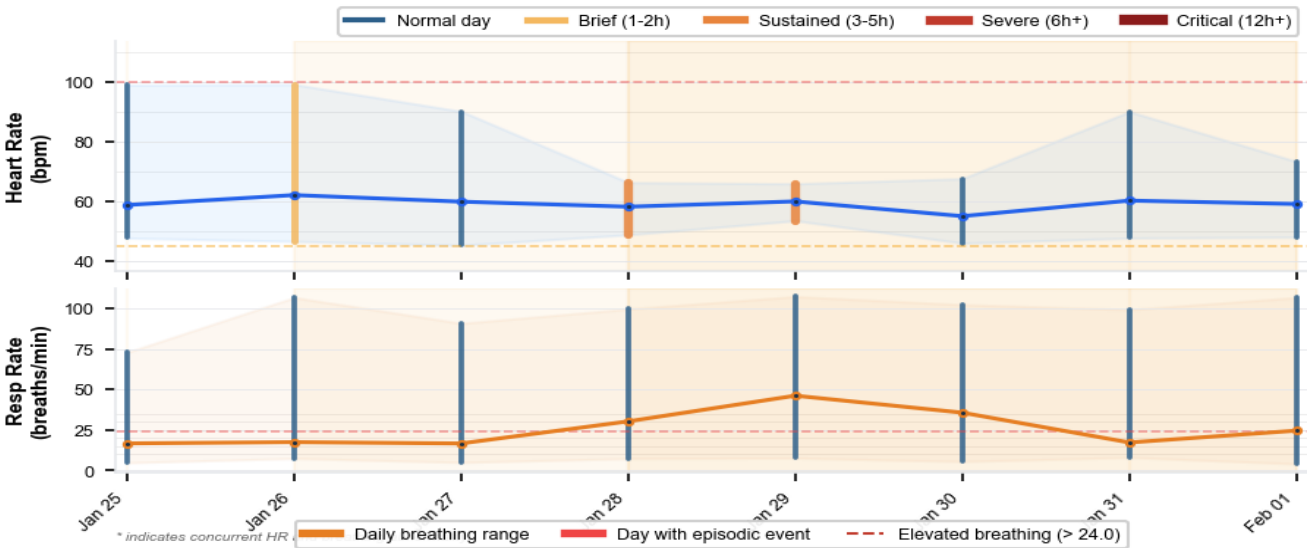


Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
25	28h	Very High Breathing	3	61%

Major Findings: Sustained very high breathing (avg 27, peak 107)

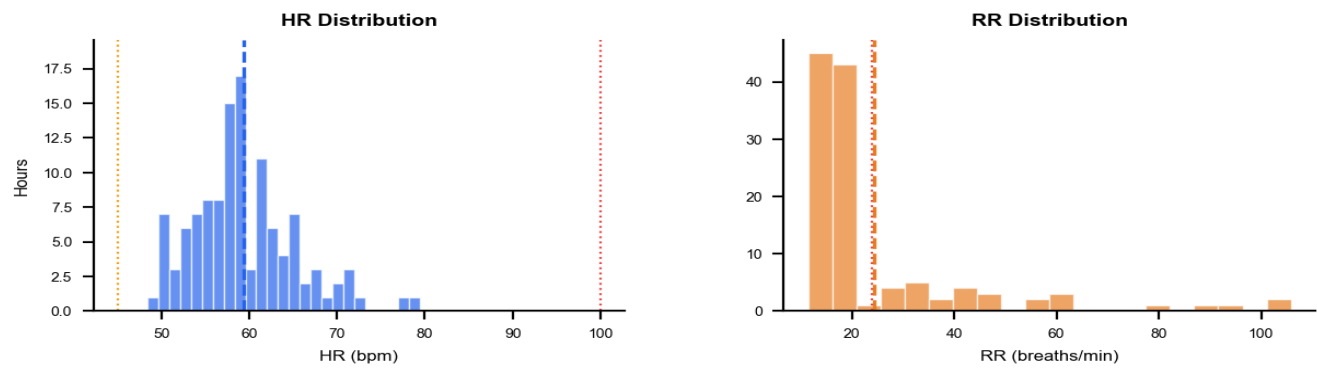
#	Episode	Min/Max	Total Hours	Longest Continuous	Episodes/day	Average	Date
1	Very High Breathing	107 brpm	17h	3h	2	27 brpm	Jan 26 to Feb 01
2	High Breathing	105 brpm	7h	1h	1	31 brpm	Jan 28 to Feb 01
3	Elevated Breathing	91 brpm	2h	2h	1	30 brpm	Jan 28
4	Elevated Breathing	35 brpm	1h	1h	1	17 brpm	Jan 25
5	Elevated Breathing	99 brpm	1h	1h	1	17 brpm	Jan 31

RR values outside physiologic range (6–60 breaths/min).

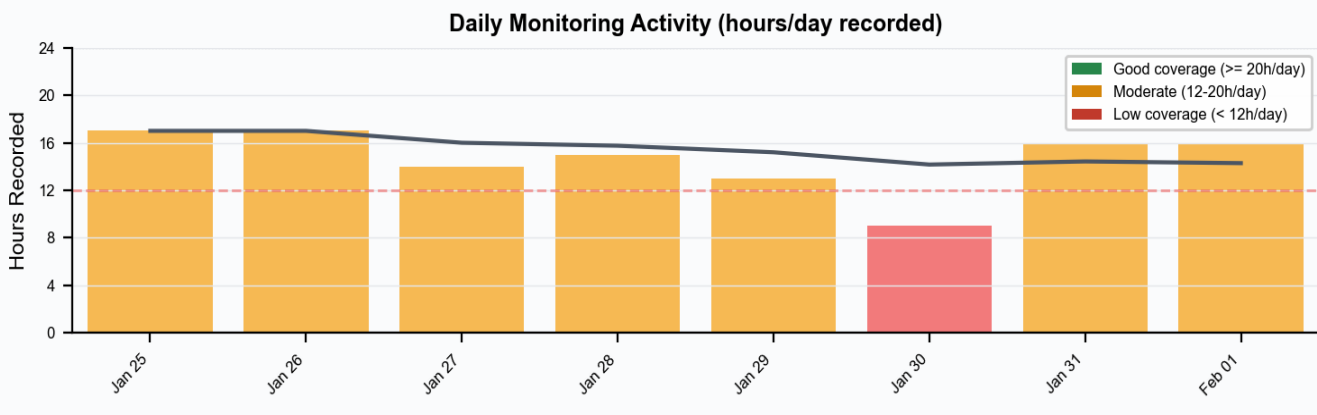


Red bars indicate days with detected episodic events.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Clinical Alerting Thresholds

■ Very Low HR < 40 bpm ■ Low HR < 45 bpm ■ Elevated HR > 95 bpm ■ High HR > 100 bpm
■ Very High HR > 110 bpm ■ Elevated Breathing > 24 brpm ■ High Breathing > 30 brpm ■ Very High Breathing > 40 brpm

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.